

STATE OF OKLAHOMA

2nd Session of the 60th Legislature (2026)

COMMITTEE SUBSTITUTE
FOR ENGROSSED
HOUSE BILL NO. 3650

By: Stinson of the House

and

Rosino of the Senate

COMMITTEE SUBSTITUTE

An Act relating to the state Medicaid program;
amending 56 O.S. 2021, Section 4002.12, as last
amended by Section 7, Chapter 448, O.S.L. 2024 (56
O.S. Supp. 2025, Section 4002.12), which relates to
minimum rates of reimbursement; extending certain
termination dates; establishing certain reimbursement
rates for multistate contracts; authorizing
promulgation of certain rules; defining terms; and
providing an effective date.

BE IT ENACTED BY THE PEOPLE OF THE STATE OF OKLAHOMA:

SECTION 1. AMENDATORY 56 O.S. 2021, Section 4002.12, as
last amended by Section 7, Chapter 448, O.S.L. 2024 (56 O.S. Supp.
2025, Section 4002.12), is amended to read as follows:

Section 4002.12. A. Until ~~July 1, 2027~~ July 1, 2028, the
Oklahoma Health Care Authority shall establish minimum rates of
reimbursement from contracted entities to providers who elect not to
enter into value-based payment arrangements under subsection B of

1 this section or other alternative payment agreements for health care
2 items and services furnished by such providers to enrollees of the
3 state Medicaid program. Except as provided by subsection I of this
4 section, until ~~July 1, 2027~~ July 1, 2028, such reimbursement rates
5 shall be equal to or greater than:

6 1. For an item or service provided by a participating provider
7 who is in the network of the contracted entity, one hundred percent
8 (100%) of the reimbursement rate for the applicable service in the
9 applicable fee schedule of the Authority; or

10 2. For an item or service provided by a non-participating
11 provider or a provider who is not in the network of the contracted
12 entity, ninety percent (90%) of the reimbursement rate for the
13 applicable service in the applicable fee schedule of the Authority
14 as of January 1, 2021.

15 B. A contracted entity shall offer value-based payment
16 arrangements to all providers in its network capable of entering
17 into value-based payment arrangements. Such arrangements shall be
18 optional for the provider but shall be tied to reimbursement
19 incentives when quality metrics are met. The quality measures used
20 by a contracted entity to determine reimbursement amounts to
21 providers in value-based payment arrangements shall align with the
22 quality measures of the Authority for contracted entities.

23 C. Notwithstanding any other provision of this section, the
24 Authority shall comply with payment methodologies required by

1 federal law or regulation for specific types of providers including,
2 but not limited to, Federally Qualified Health Centers, rural health
3 clinics, pharmacies, Indian Health Care Providers and emergency
4 services.

5 D. A contracted entity shall offer all rural health clinics
6 (RHCs) contracts that reimburse RHCs using the methodology in place
7 for each specific RHC prior to January 1, 2023, including any and
8 all annual rate updates. The contracted entity shall comply with
9 all federal program rules and requirements, and the transformed
10 Medicaid delivery system shall not interfere with the program as
11 designed.

12 E. The Oklahoma Health Care Authority shall establish minimum
13 rates of reimbursement from contracted entities to Certified
14 Community Behavioral Health Clinic (CCBHC) providers who elect
15 alternative payment arrangements equal to the prospective payment
16 system rate under the Medicaid State Plan.

17 F. The Authority shall establish an incentive payment under the
18 Supplemental Hospital Offset Payment Program that is determined by
19 value-based outcomes for providers other than hospitals.

20 G. Psychologist reimbursement shall reflect outcomes.
21 Reimbursement shall not be limited to therapy and shall include but
22 not be limited to testing and assessment.

23 H. Coverage for Medicaid ground transportation services by
24 licensed Oklahoma emergency medical services shall be reimbursed at

1 no less than the published Medicaid rates as set by the Authority.
2 All currently published Medicaid Healthcare Common Procedure Coding
3 System (HCPCS) codes paid by the Authority shall continue to be paid
4 by the contracted entity. The contracted entity shall comply with
5 all reimbursement policies established by the Authority for the
6 ambulance providers. Contracted entities shall accept the modifiers
7 established by the Centers for Medicare and Medicaid Services
8 currently in use by Medicare at the time of the transport of a
9 member that is dually eligible for Medicare and Medicaid.

10 I. 1. The rate paid to participating pharmacy providers is
11 independent of subsection A of this section and shall be the same as
12 the fee-for-service rate employed by the Authority for the Medicaid
13 program as stated in the payment methodology in OAC 317:30-5-78,
14 unless the participating pharmacy provider elects to enter into
15 other alternative payment agreements.

16 2. A pharmacy or pharmacist shall receive direct payment or
17 reimbursement from the Authority or contracted entity when providing
18 a health care service to the Medicaid member at a rate no less than
19 that of other health care providers for providing the same service.

20 J. Notwithstanding any other provision of this section,
21 anesthesia shall continue to be reimbursed equal to or greater than
22 the anesthesia fee schedule established by the Authority as of
23 January 1, 2021. Anesthesia providers may also enter into value-

1 based payment arrangements under this section or alternative payment
2 arrangements for services furnished to Medicaid members.

3 K. The Authority shall specify in the requests for proposals a
4 reasonable time frame in which a contracted entity shall have
5 entered into a certain percentage, as determined by the Authority,
6 of value-based contracts with providers.

7 L. Capitation rates established by the Oklahoma Health Care
8 Authority and paid to contracted entities under capitated contracts
9 shall be updated annually and in accordance with 42 C.F.R., Section
10 438.3. Capitation rates shall be approved as actuarially sound as
11 determined by the Centers for Medicare and Medicaid Services in
12 accordance with 42 C.F.R., Section 438.4 and the following:

13 1. Actuarial calculations must include utilization and
14 expenditure assumptions consistent with industry and local
15 standards; and

16 2. Capitation rates shall be risk-adjusted and shall include a
17 portion that is at risk for achievement of quality and outcomes
18 measures.

19 M. The Authority may establish a symmetric risk corridor for
20 contracted entities.

21 N. The Authority shall establish a process for annual recovery
22 of funds from, or assessment of penalties on, contracted entities
23 that do not meet the medical loss ratio standards stipulated in
24 Section 4002.5 of this title.

1 O. 1. The Authority shall, through the financial reporting
2 required under subsection G of Section 4002.12b of this title,
3 determine the percentage of health care expenses by each contracted
4 entity on primary care services.

5 2. Not later than the end of the fourth year of the initial
6 contracting period, each contracted entity shall be currently
7 spending not less than eleven percent (11%) of its total health care
8 expenses on primary care services.

9 3. The Authority shall monitor the primary care spending of
10 each contracted entity and require each contracted entity to
11 maintain the level of spending on primary care services stipulated
12 in paragraph 2 of this subsection.

13 P. 1. Notwithstanding the provisions of subsection A of this
14 section but subject to all other provisions of this section, for an
15 item or service delivered under a multistate contract, the
16 reimbursement rate for that item or service shall be the lesser of:

17 a. the rate specified in the multistate contract, or

18 b. the applicable rate in the fee schedule of the
19 Authority.

20 2. The Oklahoma Health Care Authority Board may promulgate
21 rules to implement the provisions of this section including, but not
22 limited to, rules that further define the terms used in this
23 subsection or that set limitations or restrictions around the
24 applicability of the provisions of this subsection.

1 3. As used in this subsection:

- 2 a. "multistate contract" means a contract that a
3 contracted entity or its parent company holds with
4 another entity under which the entity provides health
5 care services on behalf of the contracted entity
6 throughout a service area that includes this state and
7 at least one other state, and
- 8 b. "parent company" means any company that directly or
9 indirectly controls a contracted entity.

10 SECTION 2. This act shall become effective November 1, 2026.

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